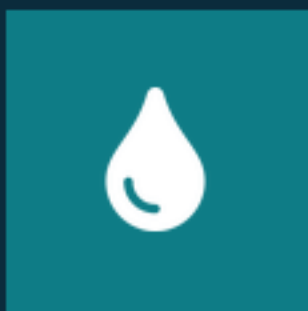




INDEPENDENT KIDNEY CARE

# A Physician-Led Dialysis Center for Metro Detroit

A partnership proposal — independent ownership, community-rooted care, real clinical autonomy.

Prepared for: [ Nephrologist / practice name ]

## THE OPPORTUNITY

# Detroit has the patients. The care model is failing them.

≈ 4×

more likely Black Americans are to reach kidney failure vs. white Americans

> 35%

of U.S. dialysis patients are Black, despite being ~13.5% of the population

~77%

of Detroit's population is Black — the exact group most affected

3×/wk

in-center treatment for years — retention & trust drive the economics

WHY NOW

## Corporate dialysis frustrates the people who keep it running.



### Physicians lose control

Two national chains run most centers. Medical directors carry the patients but get little say in how the floor is scheduled, staffed, or run.



### Staff burn out

Scheduling and planning decisions are made far from the chair. Good clinicians leave — and patient continuity leaves with them.



### Patients underserved

A revolving door of staff and rigid corporate scheduling erodes the trust that keeps patients healthy and in their seats.

**The opening: an independent center that fixes exactly what the chains get wrong.**

# Three partners. One independent center.



## You — the nephrologist

Bring the patients and serve as Medical Director. Hold meaningful equity in an independent center — not a sliver of a national chain's LLC.



## The operator

An experienced dialysis clinician who runs the floor — scheduling, staffing, planning — the way it should be run, with skin in the game.



## The investor

Funds the build-out and working capital, structures the entity, and handles the business side so clinicians focus on care.

The nephrologist is the unlock — referrals and the medical directorship anchor everything else.



## THE DIFFERENTIATOR

# A community-rooted operator is a clinical and business edge.



### Concordant care lifts retention

Patients dialyze 3× a week for years. A center led by, and reflecting, the community it serves drives the trust, engagement, and retention that protect outcomes — and revenue.



### Operations done right

Scheduling, staffing, and floor planning run by someone who has lived the work — the specific failure point that pushes patients and staff out of corporate centers.



### MWBE certification opens doors

A majority Black-woman-owned business can pursue Minority/Women Business Enterprise status — unlocking hospital supplier-diversity programs, payers, and health-equity funding.



### The strategic catch

MWBE status requires majority ownership and control by the operator. A DaVita or Fresenius JV — where the chain holds the majority — disqualifies it.

The equity advantage is only fully usable in an independent center.

## You own real equity — not a minority slice of a chain.



### Clinical autonomy

You set the standard of care and the medical directorship sits with you — no corporate playbook overriding the floor.



### Governance you shape

Ownership percentages, distributions, and decision rights are negotiated among three partners, not dictated by a national chain.



### Full upside

Profits flow to the owners by their stake — not a thin minority share after the chain takes 51-75%.

Illustrative split — set by the partners



## THE ECONOMICS

# Real margins — if you get payer mix and retention right.

~\$3M

illustrative annual receipts for an average U.S. dialysis clinic

~18%

net margin cited industry-wide — rare in healthcare



## Capital to stand one up

Build-out + water-treatment system  
Dialysis stations & medical equipment  
Life-Safety-Code-compliant facility  
Staffing + working capital through ramp  
Licensing, certification & legal structuring

## What actually drives the P&L:

Medicare pays a fixed bundled rate — same for everyone, thin on its own  
Margin is made on commercially-insured patients — payer mix is everything  
Retention compounds: every patient kept is years of stable revenue  
Home/peritoneal dialysis: lower-capital, CMS-favored growth

## THE PATH

# Michigan is a favorable place to do this.

1



### No Certificate of Need

Unlike many states, Michigan does not require a CON for a new dialysis facility — the single biggest barrier elsewhere is removed.

2



### Build & equip

Complete construction, water treatment, and Life Safety compliance. The facility must be operational before certification.

3



### CMS certification

No state license needed; you become Medicare-certified. LARA runs the survey for CMS; enroll via the MAC (Wisconsin Physician Services).

4



### Open & ramp

Onboard the nephrologist's patients, build payer mix, and grow toward home-dialysis offerings.

Source: Michigan LARA, ESRD provider guidance. Confirm current requirements with LARA + healthcare counsel.



THE ASK

# Let's explore building it together.



## A conversation

An exploratory meeting to test fit, patient volume, and shared vision.



## Your patient picture

A read on referral volume and payer mix you could anchor a center with.



## A feasibility pass

Jointly scope site, capital, and a real pro-forma with healthcare counsel.

[ Your name ] · [ email ] · [ phone ]

Not legal or financial advice. Physician-ownership dialysis JVs must be structured under Stark Law / Anti-Kickback safe harbors with qualified counsel.